Key Theories and Models of Behaviour Change Communication: Social and Community Level

1.0 Introduction to the Social and Community Paradigms

The discipline of Social and Behaviour Change Communication (SBCC) represents a sophisticated, multidisciplinary amalgamation of approaches drawn from psychology, sociology, anthropology, and communications. Historically, public health interventions and development programmes relied heavily on individual-level cognitive frameworks, such as the Health Belief Model or the Transtheoretical Model.

These early frameworks conceptualised behaviour change as an isolated, internal psychological process, if providing information to an individual would naturally precipitate a rational behavioural adjustment. However, contemporary evidence overwhelmingly demonstrates that the most successful public health programmes are those that understand health behaviours within the broader, interconnected contexts in which they occur.

When individuals make decisions regarding their health, civic participation, or environmental practices, they do not act in a socio-cultural vacuum. Their choices are profoundly constrained, facilitated, and shaped by the social networks to which they belong, the cultural norms that dictate acceptable conduct, and the structural environments that provide or deny access to essential resources.

Consequently, the field of SBCC has evolved to prioritise social and community-level theories. These theoretical frameworks offer robust strategies for intervening at the interpersonal, institutional, organisational, and regulatory levels to support and sustain collective behaviour change. Theory informs these complex programmatic debates by clarifying key constructs, articulating alternative pathways to change, and identifying the presumed causal relationships between a communication strategy and a social outcome.

This chapter provides an exhaustive exploration of the foundational principles and contemporary applications of social and community-level models in SBCC. It systematically unpacks Social Norms Theory, Diffusion of Innovations, Community Mobilisation Approaches, and Participatory Communication models, alongside an overview of the Social Ecological Model and Social Capital Theory. By examining real-world case studies documented in rigorous academic literature, the following sections will elucidate how contemporary researchers and practitioners operationalise these theories to construct environments that enable sustainable, systemic change.

2.0 Social Norms Theory

2.1 Theoretical Foundations and Core Constructs

Social Norms Theory posits that individual behaviour is fundamentally influenced by perceptions, and frequently misperceptions, of how peers think and act. Rooted in the disciplines of sociology, anthropology, and social psychology, the theory defines social norms as the informal, predominantly unwritten rules that dictate acceptable, appropriate, and obligatory actions within a specific group or society. The contemporary interest in social norms stems from a growing scholarly disillusionment with the capacity of purely information and economic inducements to reduce harmful behaviours or promote positive ones.

At the core of the contemporary application of Social Norms Theory is the taxonomy developed by Cialdini and colleagues, alongside frameworks from the Learning Collaborative to Advance Normative Change. Researchers have expanded norms into distinct, measurable constructs to better understand their influence on human behaviour.

Norm Construct	Definition within SBCC	Pragmatic Example
Descriptive Norms	An individual's cognitive belief about what other people in their specific reference group actually do.	"I believe the majority of pregnant women in my village consume iron-folic acid daily."
Injunctive Norms	An individual's belief about what others approve or disapprove of; the perceived moral or social expectations of a group.	"My community expects me to consume these supplements, and my mother-in-law will judge me if I do not."
Collective Norms	The actual, objectively measured prevalence of a behaviour or attitude within a community, as opposed to the perceived prevalence.	The statistical reality that 60% of the community consumes the supplement, regardless of public perception.

The theory relies heavily on the psychological concept of cognitive dissonance and the innate human desire for social conformity. A critical operational mechanism within this theory is that individuals frequently overestimate the prevalence of negative behaviours (such as substance abuse, gender-based violence, or school absenteeism) and underestimate the prevalence of positive, protective behaviours among their peers.

When SBCC interventions successfully correct these misperceptions by revealing the true, healthier norm, individuals naturally adjust their behaviour to align with the actual peer group standard.

Contemporary Applications in SBCC

Contemporary researchers utilise Social Norms Theory to transcend traditional information-deficit models, incorporating it into comprehensive frameworks like the Behavioural Drivers Model (BDM). Rather than merely educating target audiences about the biological consequences of a disease, SBCC practitioners now focus on mapping the normative environment through rigorous formative research, identifying who constitutes the influential "reference group," and deploying communications to alter the normative landscape.

However, contemporary literature also warns of significant methodological pitfalls in programmatic application. Practitioners often err by highlighting the overwhelming prevalence of a negative behaviour to spark urgency (e.g., broadcasting that "80% of men in this region engage in domestic violence").

According to Social Norms Theory, this inadvertently reinforces the descriptive norm that the negative behaviour is widespread and typical, potentially normalising and increasing its occurrence, a phenomenon known as the boomerang effect. Contemporary SBCC design, therefore, rigorously avoids normalising the problem. Instead, practitioners amplify the visibility of positive deviations and collective normative shifts, focusing messaging on the growing segments of society that are adopting healthy practices.

Real-World Case Studies in SBCC

Case Study 1: Voices for Change (V4C), Nigeria

The Voices for Change (V4C) programme, implemented in Nigeria between 2013 and 2017, represents a definitive, at-scale application of Social Norms Theory designed to address systemic gender inequality. Operating primarily among young adults aged 16 to 25 across four Nigerian states (Enugu, Kaduna, Kano, and Lagos), the intervention targeted three deeply entrenched normative areas: women's voice and leadership, women's roles in household decision-making, and the acceptability of violence against women and girls.

Instead of relying solely on didactic human rights messaging, researchers integrated social norms theory with sophisticated social marketing methodologies. The programme sought to shift both descriptive and injunctive norms by showcasing desirable peer behaviours.

V4C created a bespoke, youth-focused brand, utilising culturally resonant music, fashion, and digital media to frame gender equality as a normative standard inextricably linked to career success, self-fulfilment, and healthy romantic relationships. By publicly demonstrating that significant subsets of young

Nigerian men and women supported gender equality, the programme corrected the widespread misperception that discriminatory attitudes were universally endorsed.

Baseline and endline academic evaluations confirmed that addressing these structural, normative barriers through audience-centric communication significantly strengthened the enabling environment for gender equality, reducing the social sanctions associated with progressive behaviour.

Case Study 2: Iron-Folic Acid (IFA) Supplementation, India

Another robust academic application of Social Norms Theory is found in contemporary efforts to improve the uptake of Iron-Folic Acid (IFA) supplements among pregnant and lactating women in rural India. Despite decades of government supply-side interventions and mass distribution programmes, anaemia remained stubbornly prevalent.

Formative research revealed that this persistence was largely due to deeply ingrained gender meta-norms and social sanctions. Traditional gender ideologies dictated women's restricted mobility, exclusion from financial decision-making, and limited independent interactions with healthcare services.

To counter this, researchers utilised Social Norms Theory to design multifaceted interventions targeting not just pregnant women, but their primary normative influencers, specifically husbands and mothers-in-law. One prominent study tested an informed intervention utilising Facebook Ad Manager in Uttar Pradesh and Madhya Pradesh.

The intervention delivered targeted messaging to over 3 million women aimed at correcting specific normative misperceptions, such as the widely held but false descriptive norm that IFA consumption results in excessively large babies and subsequent difficult deliveries.

Concurrently, studies demonstrated that mass media exposure via radio and television was positively associated with the sustained consumption of IFA supplements for the required 100 days. These SBCC channels successfully reframed the injunctive norm surrounding maternal nutrition, establishing IFA consumption as a community expectation rather than a solitary, medically enforced task.

3.0 Diffusion of Innovations (DOI)

Theoretical Foundations and Core Constructs

Introduced by sociologist and communication scholar Everett Rogers in 1962, the Diffusion of Innovations (DOI) theory provides a comprehensive, temporal framework explaining the processes by which new ideas, processes, products, or social practices spread throughout a social system.

Unlike theories that focus exclusively on individual psychology, DOI examines the systemic channels of communication, the temporal matrices of adoption, and the structural topography of communities.

The core mechanism of DOI dictates that diffusion is a macro-level process by which an innovation is communicated through specific, trusted channels over time among members of a social system. Rogers identified five critical, perceived attributes that determine the speed and extent of an innovation's adoption by a community:

Innovation Attribute	Conceptual Definition in SBCC	Example in Public Health
Relative Advantage	The degree to which the innovation is perceived as superior to the idea, product, or practice it supersedes.	A new vaccine offering lifelong immunity compared to an older vaccine requiring annual boosters.
Compatibility	The extent to which the innovation aligns with the existing values, socio-cultural norms, past experiences, and needs of potential adopters.	Promoting plant-based diets that incorporate traditional, locally sourced ingredients rather than alien food types.
Complexity	The perceived difficulty in understanding, navigating, or utilising the new innovation.	A simple, rapid diagnostic test kit is adopted faster than a complex, multi-step laboratory procedure.
Trialability	The degree to which the innovation may be experimented with on a limited, low-risk basis before a full commitment is required.	Providing trial samples of water purification tablets before requiring households to purchase a bulk supply.
Observability	The visibility of the innovation's results to others in the social system, acting as a catalyst for peer-to-peer adoption.	Solar panels installed on the roofs of prominent community members, prompting neighbours to inquire and adopt.

Furthermore, the theory categorises the target population into a normal distribution based on their psychological and socio-economic propensity to adopt: *Innovators*, *Early Adopters* (often opinion leaders), *Early Majority*, *Late*

Majority, and *Laggards*. The cumulative adoption of an innovation typically follows an S-shaped curve (the diffusion S-curve).

This curve illustrates how people are initially slow to adopt new behaviours. However, as the behaviour becomes accepted by early adopters and observational learning occurs, a critical mass is reached, causing the early and late majority to rapidly start practising the behaviour, eventually plateauing as only the most resistant individuals remain.

Contemporary Applications in SBCC

In contemporary SBCC, DOI is instrumental in mapping the systemic spread of health technologies, agricultural practices, and preventive behaviours. Public health campaigns deliberately leverage the theory to identify "Early Adopters" and "Opinion Leaders" within communities, empowering them as formal change agents to accelerate the diffusion process through interpersonal communication.

Recent academic literature has expanded DOI by merging it with digital network analysis and spatial epidemiology. For instance, organisations map geographic and social data via Geographic Information Systems (GIS) to visually represent the diffusion of an innovation and identify precise geographic clusters where adoption has stalled.

However, modern researchers also critically evaluate the theory's inherent limitations, most notably the "pro-innovation bias". This bias is the flawed assumption that all innovations are inherently beneficial and should be adopted uniformly by all members of society, often ignoring the socio-economic barriers that structurally force marginalised groups into the "laggard" category.

Contemporary SBCC therefore couples DOI with equity-focused empowerment models to ensure innovations are financially and structurally accessible to the entire S-curve.

Real-World Case Studies in SBCC

Case Study: Insecticide-Treated Nets (ITNs) for Malaria Prevention in Sub-Saharan Africa

The diffusion of Insecticide-Treated Nets (ITNs) to combat malaria in rural African populations, extensively documented in nations such as Ethiopia and Nigeria, serves as a primary textbook application of DOI. In the early stages of ITN distribution, the adoption rate was critically slow. Formative research indicated this delay was due to cultural barriers, local misinformation regarding

the causes of malaria, and the perceived complexity of correctly hanging and maintaining the nets.

To accelerate diffusion along the S-curve, an SBCC intervention in the Oyo and Osun states of Nigeria deployed mass media alongside intense interpersonal communication channels facilitated by specifically trained "Change Agents".

These agents leveraged their status as early adopters to highlight the *relative advantage* of the nets over traditional mosquito coils and reduced perceived *complexity* through physical demonstrations.

Similarly, an academic study in the Jimma Zone of Ethiopia evaluated an intervention that utilised peer-learning mechanisms within primary schools. The SBCC strategy transformed young students into early adopters and informed communicators.

The students were taught the mechanics of ITN usage and the biological realities of malaria transmission. They subsequently returned to their households and educated their families and broader rural networks. This school-based SBCC strategy successfully exposed the adult community to malaria prevention messages through a trusted, familial channel, significantly advancing the S-curve of adoption by enhancing the *observability* and *trialability* of the nets within domestic settings.

Case Study: Direct Relief and GIS Mapping

Another contemporary application involves the non-profit organisation Direct Relief, which utilised DOI principles in conjunction with data visualisation tools to map Community Health Workers (CHWs) in Sub-Saharan Africa. By visualising dense data sets, the organisation could track the diffusion of medical supplies and the establishment of CHW networks.

This allowed SBCC practitioners to identify regions lacking early adopters and to target communication strategies to provoke behaviour change and resource allocation in highly specific locales, ultimately supporting the Sustainable Development Goals (SDGs).

3.0 Community Mobilisation Approaches

Theoretical Foundations and Core Constructs

Community mobilisation is defined within the SBCC literature as a rigorous, capacity-building process through which community individuals, groups, or

organisations plan, carry out, and evaluate activities on a participatory and sustained basis to improve their health and other collective needs.

Unlike traditional health communication campaigns, which are frequently crafted by external experts in distant urban centres and delivered to a passive rural audience, community mobilisation is fundamentally about the redistribution of social power. It is a developmental approach that is explicitly done *by* the community, rather than *to* the community.

The core of this approach lies in fostering collective action and structural group organising. Theoretical frameworks within this domain stress the importance of empowerment, which is defined as the active process by which individuals and communities gain mastery over their socio-political environments and local resources. A foundational model utilised by practitioners is the Community Action Cycle (CAC), which outlines structured, sequential phases for this empowerment process:

CAC Phase	Description of Activities in SBCC Programming
Phase 1: Prepare to Mobilise	Establishing an external facilitation team, assessing baseline community resources, and developing an ethical entry plan.
Phase 2: Community Entry & Organising	Building trust, gaining access through local leaders, inviting participation, and establishing a "Core Group" of representative local members.
Phase 3: Defining Issues & Priorities	Utilising participatory tools to collect local data, conducting root-cause analyses of health issues, and establishing a shared, communal vision.
Phase 4: Planning Together	Designing concrete strategies and facilitating sessions to create an actionable, locally owned Community Action Plan.
Phase 5: Implementation & Evaluation	Strengthening community capacity to execute the plan, monitoring collective progress, and adjusting strategies to ensure long-term sustainability.

In addition to the standard CAC, the Partnership Defined Quality (PDQ) methodology is frequently utilised. PDQ is a specific methodology designed to engage community members and institutional health service providers simultaneously. Both groups explore service quality issues individually, and then convene together to bridge quality gaps, neutralising the traditional power dynamics between medical professionals and laypeople.

Contemporary Applications in SBCC

In contemporary SBCC programming, led by prominent entities such as Save the Children and the Breakthrough ACTION consortium, community mobilisation is strategically deployed to tackle deeply entrenched structural barriers and social

stigmas. It is also highly effective during rapid-response health emergencies, such as the Ebola or COVID-19 outbreaks, where top-down messaging often fails due to a lack of institutional trust.

Contemporary researchers utilise these frameworks to ensure programmatic sustainability; because the interventions fundamentally build local capacity, established community coalitions often evolve into independent local non-governmental organisations that continue the work long beyond the project's original life cycle. Activities range from intimate door-to-door dialogue sessions to the utilisation of community spokespeople who act as positive role models to decrease stigma.

Real-World Case Studies in SBCC

Case Study: Tostan and the Abandonment of Female Genital Mutilation (Senegal)

The Tostan Community Empowerment Programme in Senegal stands as one of the most rigorously evaluated and internationally celebrated examples of community mobilisation in global public health. The programme targeted the highly entrenched, culturally sensitive practice of Female Genital Mutilation/Cutting (FGM/C) alongside early child marriage.

Recognising that top-down, biomedical messaging dictating the dangers of FGM was frequently met with profound cultural resistance, Tostan initiated a multi-year, community-led mobilisation strategy based on non-formal adult education and basic human rights. The methodology involved training local facilitators who lived within the communities and led ongoing, respectful dialogues about democracy, problem-solving, hygiene, and human rights.

A critical element of Tostan's success was its sophisticated understanding of the social network dependency of FGM. Formative research revealed that families practiced FGM primarily to ensure their daughters' marriageability within a specific, extended network of surrounding villages. Therefore, if a single, isolated village abandoned the practice, their daughters would be rendered unmarriageable, resulting in severe social and economic sanctions.

Tostan's mobilisation approach addressed this macro-structural barrier by facilitating "inter-village meetings". The empowered core groups from the initial intervention sites travelled to neighbouring villages, engaging their extended kinship and marriage networks in the human rights dialogue.

Through sustained dialogue, when a critical mass of interconnected villages reached a collective consensus, they participated in highly publicised,

community-organised public declarations of abandonment. This public, collective action dismantled the prevailing social convention simultaneously across the entire geographic network, effectively neutralising the social risk of abandonment.

The Tostan case study conclusively demonstrates that when community mobilisation generates a critical mass of informed, empowered citizens, profound and seemingly intractable social transformations can be achieved and sustained.

4.0 Participatory Communication and Empowerment Models

Theoretical Foundations and Core Constructs

Participatory communication represents a radical, philosophical departure from the traditional transmission model of communication. The traditional approach is often termed the "megaphone" model, wherein information flows linearly from an authoritative sender (e.g., a Ministry of Health) to a presumed passive receiver.

In stark contrast, participatory communication functions as a "roundtable" dialogue. It is conceptualised as a two-way, horizontal process where information, indigenous knowledge, and diverse perspectives are shared equitably.

The intellectual roots of this model draw heavily from critical pedagogy, specifically the seminal works of Paulo Freire, centring on the concept of 'critical consciousness'. In this paradigm, the target community is deeply involved at every stage of a development project: from identifying the structural and political causes of their vulnerabilities, to designing, implementing, and evaluating the solutions. The goal of this model is not mere behavioural modification, but rather *cognitive and relational empowerment*.

Cognitive empowerment involves community members gaining a critical understanding of their sociopolitical reality and recognising their own agency. Relational empowerment involves levelling structural power differentials, enabling marginalised groups to negotiate effectively with institutional actors, secure resources, and demand policy changes.

Contemporary Applications in SBCC

In modern SBCC, participatory models are institutionalised through frameworks such as Community-Based Participatory Research (CBPR). These frameworks emphasise the co-creation of knowledge between academic researchers and community members, explicitly addressing the inherent power dynamics that

traditionally elevate professional, biomedical knowledge over lived community experience.

However, contemporary communication scholars present critical theoretical cautions regarding the practical implementation of these models. Recent academic literature rigorously interrogates the risk of "tokenism".

Critical theorists argue that participatory methods are frequently co-opted by institutional actors (such as large NGOs or governments) to maintain control while merely simulating inclusivity. When participation is depoliticised and employed as a managerial tool to secure compliance for pre-determined agendas, rather than as a transformative force for power redistribution, it reinforces hierarchical structures and creates an "illusion of empowerment".

Consequently, robust SBCC research today demands profound reflexivity and authentic power-sharing mechanisms to ensure participation translates into genuine structural change rather than rhetorical compliance.

Real-World Case Studies in SBCC

Case Study 1: Soul City Institute for Health and Development Communication (South Africa)

The Soul City Institute in South Africa represents a paradigm-shifting application of participatory communication embedded within a massive Entertainment-Education (Edutainment) vehicle. Originating in the post-apartheid era, the intervention was designed to address pressing public health emergencies, specifically the HIV/AIDS epidemic and pervasive domestic violence, through a synergistic blend of mass media and grassroots community participation.

Soul City Series 4 provides a masterclass in participatory methodology at a national scale. The core multimedia vehicle comprised a 13-part prime-time television drama, a 45-part radio drama broadcast in nine official languages, and the distribution of millions of full-colour print booklets.

However, rather than dictating behavioural directives from a studio, the narratives were formulated through exhaustive participatory research involving the target communities. This ensured that the characters modelled plausible, culturally resonant coping strategies and pro-social behaviour rather than patronising solutions.

Crucially, the communication strategy was horizontally integrated with civil society through a formal partnership with the National Network on Violence Against Women (NNVAW), a coalition of over 1,500 local activists, government

representatives, and community groups. The media broadcasts served to spark interpersonal dialogue at the community level, while the participatory coalition channelled this heightened awareness into direct community action and political advocacy.

The academic impact evaluation of Soul City 4, comprising national surveys, sentinel site studies, and qualitative assessments, demonstrated profound empowerment outcomes. It successfully linked vulnerable individuals to the newly established Stop Women Abuse Helpline (SWAH), overwhelming the initial infrastructure with demand.

Furthermore, it generated massive community-level advocacy that expedited the provincial implementation of the national Domestic Violence Act (DVA) in 1999. By combining the vast reach of edutainment with the deep, relational empowerment of participatory community networks, Soul City successfully stimulated interpersonal dialogue, shifted normative behaviours, and enacted systemic policy change.

Case Study 2: Communities for Healthy Living (CHL)

A further academic example of participatory models in action is the Communities for Healthy Living (CHL) initiative, a family-centred childhood obesity prevention intervention integrated into Head Start services. This case study directly applied Empowerment Theory to the CBPR process to mitigate tokenism.

Recognising the power differentials between low-income parents, Head Start teachers, academic researchers, and administrators, the programme formalised equitable decision-making and resource-sharing. By integrating cognitive and relational empowerment into the conceptual logic model, the intervention ensured that the parents, who are often impacted by profound social determinants of health, held equal weight in designing the dietary and physical activity interventions as the professional academics.

5.0 Other Relevant Social and Community-Level Theories

To establish a truly exhaustive understanding of SBCC, scholars and practitioners must look beyond strictly communicative and mobilisation processes to theories that map the broader structural, environmental, and relational ecosystems. Two highly relevant frameworks frequently integrated into contemporary SBCC are the Social Ecological Model and Social Capital Theory.

The Social Ecological Model (SEM)

The Social Ecological Model (SEM) is a theory-based framework that conceptualises human health and social behaviour as the product of complex interplays between an individual and their multi-layered environment. SEM fundamentally challenges interventions that place the sole burden of change on the individual, arguing that behaviour is conditioned by multiple, nested spheres of influence.

The literature generally defines these ecological layers as follows:

SEM Layer	Focus within SBCC Strategy	Examples of Interventions
Individual/Intrapersonal	Knowledge, attitudes, biological traits, and self-efficacy skills.	Educational lessons; self-management video games.
Interpersonal	Family networks, friends, and peer dynamics providing social support.	Discussion groups with parents; peer mentoring programmes.
Community	The social networks and structural designs of neighbourhoods, including access to safe spaces.	Community-wide multimedia campaigns; network mapping.
Organisational/Institutional	Schools, workplaces, and local health services that dictate daily routines.	Altering choice architecture in school cafeterias.
Policy/Societal	National laws, economic structures, mass media framing, and cultural macro-narratives.	Advocacy for sugary drink taxes; establishing domestic violence acts.

Application in SBCC: Childhood Obesity Prevention

The SEM is extensively utilised in SBCC strategies targeting complex, non-communicable epidemics such as childhood obesity. Contemporary researchers recognise that educating a child or parent about healthy eating (intervening solely at the individual level) is entirely futile if the surrounding environment is a "food desert" devoid of nutritional options, or if the built environment lacks safe outdoor spaces for physical activity.

A multi-component SBCC intervention utilising SEM coordinates efforts across all layers. For example, researchers might deploy educational resources for children (Individual), while simultaneously facilitating discussion groups for parents regarding home cooking (Interpersonal). Concurrently, they might implement community-wide multimedia campaigns to promote outdoor play (Community). A critical intervention often occurs at the organisational level; for

instance, a study in secondary schools demonstrated that altering the "choice architecture" of cafeterias (placing healthier foods in prominent positions) significantly increased the selection of salads and sandwiches, proving that environmental adjustments drive individual choice.

Finally, advocacy campaigns pressuring local governments to regulate fast-food zoning or implement sugary drink taxes address the macro-environment (Policy). By synchronising SBCC across these ecological layers, practitioners address both the internal behavioural drivers and the external structural barriers to healthy living.

Social Capital Theory

Social Capital Theory explores the inherent value generated by social networks and the inclinations that arise from these networks to perform actions for mutual benefit. Drawing significantly from the sociological concepts of Pierre Bourdieu, the theory argues that social relationships are literal resources—capital—that can lead to the accumulation of human capacity, social mobility, and the facilitation of cooperative action.

Contemporary researchers divide social capital into two critical dimensions highly applicable to SBCC design:

- **Individual Social Capital (ISC):** The resources an individual can access through their personal networks, relying on social support, network learning, and social identity.
- **Collective Social Capital (CSC):** The aggregate cohesion, trust, and shared institutional norms within a community that allow for collective problem-solving and systemic resilience.

Application in SBCC: Environmental Behaviour and the Water-Health Nexus

In the context of SBCC, Social Capital Theory is frequently applied to sustainability, environmental health, and anti-corruption campaigns. For instance, disaster risk reduction in urban environments often depends on collective action.

An academic study mapping collective social capital in Kampung Gambiran, Indonesia, demonstrated how communities leveraged deep-seated network trust to organically form environmental action groups that successfully mitigated annual disaster risks.

Furthermore, researchers argue that SBCC campaigns aimed at reducing specific public health risks—such as substance abuse among pregnant women—frequently fail if they only seek to increase general biomedical knowledge. Practitioners must tap into the local social capital, utilising existing ecological and cognitive networks to organically motivate change through trusted relationships. By formally mapping the presence (or absence) of communal trust and civic engagement, SBCC practitioners can design interventions that either harness strong existing networks or seek to build "bridging capital" where community fragmentation prevents the diffusion of healthy practices.